

MEQ PRACTICE QUESTIONS (CASE STUDIES)

MODIFIED ESSAY QUESTION 3

You have recently assessed Mr Newbury, aged 84 years, who lives in residential care centre for people with dementia. Mr Newberry suffers from moderately severe Alzheimer's disease. The staff are concerned that he is verbally aggressive towards staff and other residents and occasionally lashes out and hits staff. Some of the untrained staff in the centre feel that Mr Newberry is deliberately playing up and are annoyed with him.

When you examine Mr Newberry, he is calm but severely cognitively impaired. He has no apparent physical problems. Recent routine lab investigations including midstream urine culture are unremarkable. Mr Newberry is on no medication.

a) Describe the information you would ask residential care staff to collect regarding Mr Newberry's difficult behaviour.

When you return to the centre one week later, you find that staff have not collected any information for you.

b) Discuss the factors that make it difficult for care staff to collect such information

The management of the centre asks for a teaching session for the regular staff on management of difficult behaviour and dementia patients.

c) Discuss how you would educate the staff?

MODIFIED ESSAY QUESTION 4

Omar is a 23-year-old gentleman living at home with both his parents who are medical practitioners. He has been diagnosed with bipolar affective disorder and has a history of cannabis and methamphetamine dependence. He has been treated under CTOs twice previously where he lived in another city and is on risperidone and lithium. He has now moved back with his parents under the condition that he takes his medications. Parents are prescribing and checking his levels.

a) What information do you need before assessing Omar?

His father calls the clinic and states that he does not wish Omar to be seen by the registrar even though Omar finds the registrar useful. He wants the son to be reviewed by the consultant.

b) How would you outline your approach to the request?

Father wants Omar to have more intervention and feels that not enough is being done for him. He has read about IPSRT and wants to find out how this would be helpful for Omar.

c) Describe what it is and how it might benefit the patient.

Father has now prescribed and administered a depot.

d) How would you handle the situation?

MODIFIED ESSAY QUESTION 5

You are a consultation liaison psychiatrist. You are referred Bill by the endocrinology team. Bill is a 45-year-old married electrician who has a history of Pituitary carcinoma. His tumour was successfully removed by the endocrinology team. However, since that time, Bill has developed mood lability, social withdrawal and low self-esteem. He initially returned to work as an electrician, however, left his work after 6 months as he lost his confidence. He also suffers from chronic headaches and insomnia. Bill has sleep apnoea and uses a CPAP machine. He is managed with thyroxine and corticosteroids.

a) Outline the reasons for Bill's mood lability (6 marks)

You meet with Bill and his wife. Bill's wife has taken over cleaning, shopping and management of bills since Bill's surgery. Bill feels angry and frustrated about this. According to the occupational therapist, Bill is capable of doing these things himself.

b) Outline the management of the situation.

The endocrinology team tell you that Bill's tumour has returned although it is treatable. Bill has become increasingly irritable and labile in his mood.

c) Outline the possible causes for Bill's mental state deterioration (4 marks)